



Veterans Administration

VHA FAX TRANSMITTAL

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**Audie L. Murphy VA Hospital,
STVHCS**

7400 Merton Minter
San Antonio, TX 78229
210-617-5300

DATE:	Apr. 9, 2024
ATTN:	
FROM:	Community Care
SUBJECT:	Referral

Note:

Please see referral for review and scheduling



U.S. Department
of Veterans Affairs

**VA Form 10-7080 - Approved Referral For
Medical Care**

Veteran Name: Kelley, Marvin Stephen

Veteran ICN: 1014557320V521981

Veteran EDIPI: 1346873406

Veteran Date of Birth: 1950-03-18

Veteran Preferred Name:

Pronoun:

Pronoun Description:

Veteran Address: 705 REASER DR
VICTORIA, TX 77905

Veteran Phone Number: (361)645-9170

Veteran Mobile Phone Number (if Known): (999)999-9999

Veteran Business Phone Number (If Known): (361)645-9501

Veteran Email Address (If Known): mjkelley47@hotmail.com

Referral Number: VA0036503610

Priority: Routine

Referral Issue Date: 2024-03-20

Expiration Date: PRELIMINARY 2024-09-16 (SEE BELOW)*

First Appointment Date: SUPPLY TO VA ASAP

Referring VA Facility: Audie L. Murphy Memorial Veterans' Hospital

VA Telephone Number: 210-949-3850

VA Fax Number:

Initial Community Care Provider/Facility: VICTORIA DERMATOLOGY PA

Initial Provider Location: VICTORIA DERMATOLOGY PA-115 Medical Dr ; Ste 202, Victoria, TX, 77904-207N00000X

Provider Name (if known): PATEL, AMIT

Provider Phone: 281-836-4690

Provider Fax: 409-729-2449

Community Provider NPI: 1346553120

Caregiver Type: Individual caregiver

Caregiver Details: Mary Jane Keller (Spouse) 361

***IN ORDER TO PROVIDE THE FULL DURATION OF THE STANDARDIZED EPISODE OF CARE (SEOC)
PLEASE CONTACT THE REFERRING VA FACILITY WITH A FIRST APPOINTMENT TO CALCULATE
A FINAL EXPIRATION DATE. CLAIM PAYMENTS ARE DEPENDENT UPON THESE DATES.**

Any claim related to this episode of care **MUST INCLUDE THE APPROVED REFERRAL NUMBER** as the
Referral Number or Prior Authorization number.

**Please see below for Additional VA Referring Facility Information, Billing Information
and Appointment/Provider Information.**

Pertinent Clinical Information

**Please view the Clinical Information in the VA Order section for more information related to the Original VA Order
Reason for Request.**

Chief Complaint: skin changes and multiple nevis

Patient History / Clinical Findings / Diagnosis (Co-Morbidities): skin changes and multiple nevis Is this referral for
Service Connected condition? No Per VA PBM Formulary Process Management Policy from VHA Handbook 1108.08, the
use of drugs solely to improve normal physiologic function or to enhance body appearance is generally not considered

medically necessary and therefore are not to be prescribed. Medications used for this purpose are considered cosmetic. The use of medications for hair re-growth (e.g., minoxidil, finasteride, or pimecrolimus) are considered cosmetic in nature and should not be prescribed unless care is being provided in connection with the treatment of a service- connected injury.

Provisional Diagnosis: R239 Unspecified skin changes

Services Authorized

The VA Order Reason for Request is the official clinical order. This scope of services associated with the medical care for this authorization is found below. Necessary services that are not included must be requested using the Request for Services procedures. Please visit the VHA Storefront www.va.gov/COMMUNITYCARE/providers/index.asp for additional resources and requirements.

Service Requested: Dermatology_PRCT SEOC 1.0.12

Category of Care: DERMATOLOGY

Procedural Overview - Standardized Episode of Care (SEOC)

Dermatology_PRCT SEOC 1.0.12 Duration: 180 Days

Description:

This authorization covers services associated with the specialty(s) identified for this episode of care, including all medical care listed below relevant to the referred care specified on the consult/referral order.

NOTE: Please note this SEOC does not cover cosmetic procedures that are not considered medically necessary or correct a functional disability as they are excluded from the Medical Benefits Package.

NOTE: This SEOC does not cover Mohs. A separate referral specifically for Mohs with the Mohs SEOC is required.

NOTE: Superficial Radiation Therapy (SRT) is not covered in this episode of care. SRT is only to be performed and approved by a radiation oncologist and not a dermatologist. SRT must be requested through an RFS and approved using one of the radiation therapy SEOCs.

NOTE: Additional consultations needed relevant to the patient complaint/condition require VA review and approval including a referral to another specialty for secondary/complex closure.

NOTE: This SEOC does not cover the removal of skin tags, seborrheic keratosis, and other benign skin lesions unless they are bleeding, painful, or changing.

No.	Service/Procedure	Number Of Visits Authorized
1	Initial outpatient evaluation (including full body exam), treatment, and follow-up visits for the referred condition on the consult/referral order	999
2	Diagnostic imaging relevant to the referred condition on the consult/referral order	999
3	Labs and pathology relevant to the referred condition on the consult/referral order	999
4	Procedures performed by the dermatology provider relevant to the referred condition on the consult/referral order including but not limited to: cryotherapy, biopsy (e.g., excisional,	999

shave, punch), excision, repair, excimer laser, photodynamic therapy, UVB therapy, debridement, medically necessary hair removal

Additional Information:

- * Please visit the VHA Storefront www.va.gov/COMMUNITYCARE/providers/index.asp for additional resources and requirements pertaining to the following:
- * Pharmacy prescribing requirements
- * Durable Medical Equipment (DME), Prosthetics, and Orthotics prescribing requirements
- * Precertification (PRCT) process requirements
- * Request for Services (RFS) requirements

REFER ALL QUESTIONS RELATED TO THIS APPROVAL TO THE ISSUING VA OFFICE

Referring VA Facility: Audie L. Murphy Memorial Veterans' Hospital

Station Number: 671

Telephone Number: 210-949-3850

Address: 7400 Merton Minter Boulevard SAN ANTONIO TX 78229

Referring Provider: 1922510320

Referring Provider NPI: 1922510320

Unique Consult No: 671_8511327

Program Authority: Authorized/Pre-authorized VA Referral (not otherwise specified) - 1703

Affiliation: Triwest

Network: CC Network 4

Appointments/Providers Assigned to the Referral

Provider/Facility Name	Provider/Facility Location	Appt Date	Appt Time	Telephone #	Fax #
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Additional Service Information**Request for Services (RFS)**

A Request for Services (RFS) is a provider-generated request for new or additional care outside the scope of the current approved referral/authorization. Provider should always submit the RFS directly to the authorizing VAMC, preferably via the HSRM portal. Providers should always submit an RFS on the same day it is determined it's needed and before delivering care, unless it is emergent care. In that case, the RFS can be submitted simultaneously.

How to Submit an RFS

VA prefers providers submit an RFS via the HSRM portal, available on the VA's website

- Go to the VA Storefront at <https://www.va.gov/COMMUNITYCARE/providers/Care-Coordination.asp#RFS>
 - Navigate to the link to the RFS form at the bottom of the section

Medical Records and Documents Requirements

Medical Records and documentation are required for all provider services. Providers are required to submit Medical documentation directly to the authorizing VAMC, preferably via upload to HSRM.

Billing and Other Referral Information**Submitting Claims**

ANY CLAIMS RELATED TO THIS EPISODE OF CARE MUST BE SUBMITTED TO TRIWEST OR DELTA DENTAL AS APPROPRIATE AND INCLUDE THE APPROVED REFERRAL NUMBER.

Methods to submit claims:**Electronic Data Interchange (EDI):**

Payer ID for Medical – TWVACCN

Payer ID for Dental – CDCA1

More information on how to submit claims can be found by visiting

<https://www.va.gov/COMMUNITYCARE/revenue-ops/Veteran-Care-Claims.asp>

Precertification

The Standardized Episode of Care (SEOC) referral you have accepted does not include services that require third-party payer (TPP) precertification.

Community Care Medical Policies

VA Community Care Medical Policies describe standard VA health care benefits for services and procedures that community providers may recommend as necessary for a Veteran. Prior to providing care, providers should use the Community Care Medical Policies as a reference when determining if a Veteran meets VA clinical criteria. When additional services are requested, Community Care Medical Policies will be used to determine approval by a clinical reviewer. Community Care Medical Policies and supporting information can be found at:

<https://www.va.gov/COMMUNITYCARE/providers/info-CDI.asp>

Medical Devices

As part of furnishing the services authorized on the approved referral, you may recommend that medical devices, adaptive equipment, or other items be provided for the treatment or rehabilitation of the Veteran's medical condition. In such instances, you must complete a Request for Services (RFS) as outlined above.

<https://www.va.gov/COMMUNITYCARE/providers/Care-Coordination.asp#RFS>

The RFS must (1) indicate that the condition for which the item is being prescribed is within the scope of the authorized services on the approved referral, (2) describe the item or service being prescribed with as much specificity as possible (including manufacturer and model, needed custom measurements, size), and (3) provide a brief but thorough plain-language explanation of how the item or service will serve the treatment or rehabilitation needs of the Veteran. All parts of the RFS form must be filled out completely.

VA staff will determine whether the prescribed item or service is one that VA is authorized to purchase. If the item is not something that we are authorized to purchase, VA staff will work with you to identify an alternative that will best meet the Veteran's clinical needs. In making this determination, VA staff will consider your clinical judgment and expertise and work with you to resolve outstanding issues quickly.

In the event of an urgent or emergent need, you may provide medical devices, necessary items or services prescribed to Veterans receiving care in the community for urgent or emergent conditions at the time of healthcare service delivery or soon thereafter. Urgent or emergent DME or Medical Devices may include, but are not limited to splints, crutches, canes, slings, soft collars, walkers, and manual wheelchairs. DME dispensed directly to the Veteran based on an urgent or emergent need will be paid by the CCN Third Party Administrator. Claims for these items should be submitted using the process outlined in the **Submitting Claims** section of this form.

Pharmacy**Drug Safety and Administration Requirements**

• Must follow the VA National Protocol and clinical guidance for Esketamine or Ketamine administration. Ketamine treatments for mental health or for pain are not approved under a CCN referral. Prior to administration it is required the ordering and administering providers review the VA Protocol and Clinical Guidance found through the VA Formulary Search Tool available here:

• <https://www.pbm.va.gov/apps/VANationalFormulary/>

Must follow the VA Opioid Safety Guidelines and complete the required Opioid Safety Training found here:

• <https://www.pbm.va.gov/apps/VANationalFormulary/>

• <https://www.va.gov/COMMUNITYCARE/providers/EDU-Training.asp>

CVS Caremark is the retail pharmacy network for Veterans' immediately needed or Urgent/Emergent prescriptions.

Immediate need prescriptions:

- Must follow the VA Urgent/Emergent Formulary which can be found at <http://www.pbm.va.gov/PBM/nationalformulary.asp>
- Prescription can only go up to a 14-day supply. No refills of the immediate need medication may be authorized.
- Only a seven-day supply for opioids, or up to the opioid prescribing limit allowed by State—whichever is less—may be authorized.

Immediate need prescription extending past 14 days:

- The provider will need to send second prescription (beyond 14 days) to the referring VA medical facility's pharmacy for prescription fulfillment services.

Routine/maintenance prescriptions:

- Must be sent to the referring VA medical facility's pharmacy

If you do not have the ability to electronically submit prescriptions to pharmacies, please contact the Community Care representative at the referring VA medical facility for their pharmacy fax number. Please refer to: <https://www.va.gov/COMMUNITYCARE/providers/Service-Requirements.asp> for additional instructions related to prescriptions.

Clinical Information on the VA Order

Reason for Request:

Provider to receive results:NP purple

Type of Service: Evaluation and Treatment

skin changes and multiple nevis

Chief Complaint: skin changes and multiple nevis

Patient History / Clinical Findings / Diagnosis (Co-Morbidities):

skin changes and multiple nevis

Is this referral for Service Connected condition? No

Per VA PBM Formulary Process Management Policy from VHA Handbook 1108.08, the

use of drugs solely to improve normal physiologic function or to enhance body

appearance is generally not considered medically necessary and therefore are not

to be prescribed. Medications used for this purpose are considered cosmetic.

The use of medications for hair re-growth (e.g., minoxidil, finasteride, or

pimecrolimus) are considered cosmetic in nature and should not be prescribed

unless care is being provided in connection with the treatment of a service-

connected injury.

****This 10-7080 - Approved Referral For Medical Care was generated on 04/09/2024 changes made to the referral after this date are not reflected on the form.**



U.S. Department of Veterans Affairs

Veterans Health Administration
Office of Community Care

Health Share Referral Manager (HSRM): VA's Secure Online Portal for Managing Referrals and Authorizations

Why are community providers across the country excited to use HSRM?

- ◆ **Facilitates Health Information Exchange (HIE)** between community providers and VA via a unified platform
- ◆ Conveniently **organizes all active referrals** in one centralized location
- ◆ **Reduces time spent** waiting for fax, phone or email contact
- ◆ Allows community providers to **request authorization** for additional services or additional time to provide services
- ◆ Promotes **reduced turnaround time** for authorizations and reimbursement

HSRM Account Creation For Community Providers

- ◆ **STEP 1: Training:** Each staff member attends virtual training, completes eLearning lessons, or reviews one of the training guides
- ◆ **STEP 2: ID.me:** Each staff member creates an ID.me account and verifies their identity at <https://www.id.me/>
- ◆ **STEP 3: Submit EUT:** One facility point of contact (POC) fills out the End User Tracker (EUT), then sends it to HSRMSupport@va.gov
- ◆ **STEP 4: Receive Accounts:** Help Desk creates accounts in HSRM, then provides confirmation of account creation to the facility POC
- ◆ **STEP 5: Log Into HSRM:** Each staff member logs into HSRM at <https://ccracommunity.va.gov>
- ◆ Once these steps are complete, contact the VA Medical Center(s) you work with to let them know you have access to HSRM and to discuss your transition to HSRM

HSRM Registration Resources

- ◆ HSRM Support Points of Contact: <https://www.va.gov/COMMUNITYCARE/providers/HSRM-POC-List.asp>
- ◆ ID.me Registration Instructions: https://www.va.gov/COMMUNITYCARE/docs/pubfiles/factsheets/FactSheet_26-06.pdf#
- ◆ End User Tracker
 - End User Tracker Template: https://www.va.gov/COMMUNITYCARE/docs/providers/HSRM_End_User_Tracker.xlsx#
 - Send completed End User Tracker to HSRMSupport@va.gov
- ◆ HSRM Login: <https://ccracommunity.va.gov>

HSRM Educational Resources

- ◆ Live Virtual Training: https://www.train.org/virginia/course/1082953/live_event; please visit link to find available dates and to register in advance
- ◆ HSRM eLearning lessons: Log into <https://www.train.org/vha/welcome>, then search the course catalogue for "HealthShare Referral Manager"
- ◆ HSRM User Guide: https://www.va.gov/COMMUNITYCARE/docs/providers/HSRM_User_Guide.pdf
- ◆ HSRM Quick Reference Guide (QRG): https://www.va.gov/COMMUNITYCARE/docs/providers/HSRM_Quick_Reference_Guide.pdf#
- ◆ Additional information about HSRM: https://www.va.gov/COMMUNITYCARE/providers/Care_Coordination.asp
- ◆ HSRM Help Desk
Phone: 1-844-293-2272, Email: HSRMSupport@va.gov, Hours: Monday - Friday 7:30 a.m. - 5 p.m. Eastern Time



Veterans Health Administration

VHA Medical Documentation

Document Created: 04/09/2024 19:44

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AUDIE L MURPHY MEMORIAL VAMC 7400 Merton Minter Blvd, San Antonio, Texas 78229-4404
VA Referrals phone: 210-949-3850 Fax:

Point of Contact (POC): MARTINEZ, GRETCHEN
POC Phone: 2109493850 Fax:

Patient Name: KELLEY,MARVIN
STEPHEN

DOB: 03/18/1950

705 REASER DR
VICTORIA, TEXAS 77905

Phone (residential): (361) 645-9170
Phone (mobile): (999)999-9999

Referral Type: Community Care
Network

Next of Kin contact information

KELLEY,MARY JANE

Address (Next Of Kin)
705 REASER DR
VICTORIA, TEXAS 77905

Phone: (361)645-9170
Phone (work): NONE

CONSULTS

Current PC Provider: COLLIS,LAURIE C
Current PC Team: VICTORIA PURPLE TEAM *WH*
Current Pat. Status: Outpatient
Primary Eligibility: SERVICE CONNECTED 50% to 100%
UCID: 671_8511327
Patient Type:
OEF/OIF: NO

Order Information

To Service: COMMUNITY CARE-DERMATOLOGY
Attention:
From Service: VOP PACT NP
Requesting Provider: COLLIS,LAURIE C
Service is to be rendered on an OUTPATIENT basis
Place: Consultant's choice
Urgency: Routine

Clinically Ind. Date: 2024-04-02 00:00:00

DST ID:

Orderable Item:

Consult: Consult

Provisional Diagnosis: Unspecified Skin Changes (ICD-10-CM R23.9)

Reason For Request:

Provider to receive results:NP purple

Type of Service: Evaluation and Treatment

skin changes and multiple nevis

Chief Complaint: skin changes and multiple nevis

Patient History / Clinical Findings / Diagnosis (Co-Morbidities):

skin changes and multiple nevis

Is this referral for Service Connected condition? No

Per VA PBM Formulary Process Management Policy from VHA Handbook 1108.08,

the use of drugs solely to improve normal physiologic function or to enhance body appearance is generally not considered medically necessary and therefore are not to be prescribed. Medications used for this purpose are considered cosmetic. The use of medications for hair re-growth (e.g., minoxidil, finasteride, or pimecrolimus) are considered cosmetic in nature and should not be prescribed unless care is being provided in connection with the treatment of a service- connected injury.

Inter-facility Information

AUDIE L. MURPHY MEMORIAL HOSP

Status: ACTIVE

Last Action: RECEIVED

Facility

Activity Date/Time/Zone Responsible Person Entered By

CPRS RELEASED ORDER 03/19/24 12:41 COLLIS,LAURIE C COLLIS,LAURIE C

ADDED COMMENT 03/19/24 00:00 COLLIS,LAURIE C COLLIS,LAURIE C

RECEIVED 03/20/24 08:04 MANLEY,COLEEN N MANLEY,COLEEN N

Note: TIME ZONE is local if not indicated

No local TIU results or Medicine results available for this consult

=====

===== END =====

PROBLEM LIST

Sensitive Diagnoses

No sensitive diagnoses were provided.

Other Diagnoses

Problem	Code
Calculus of bile duct w/o cholangitis or cholecyst	K80.50
CELLULITIS OF ARM	682.3
Cervicalgia	M54.2
Chronic obstructive pulmonary disease, unspecified	J44.9
Contact with and exposure to other hazardous subst	Z77.29
CYSTITIS NOS	595.9
Encounter for therapeutic drug level monitoring	Z51.81

Essential (primary) hypertension	I10.
Gastro-esophageal reflux disease without esophagit	K21.9
Hyperlipidemia, unspecified	E78.5
Hypothyroidism, unspecified	E03.9
Low back pain	M54.5
Malignant neoplasm of prostate	C61.
Myalgia	M79.1
Obstructive sleep apnea (adult) (pediatric)	G47.33
Other seborrheic keratosis	L82.1
Other specified arthritis, left wrist	M13.832
Other vitamin B12 deficiency anemias	D51.8
Patients noncompliance w oth medical treatment an	Z91.19
Post-traumatic stress disorder, chronic	F43.12
Spondylosis w/o myelopathy or radiculopathy, cervi	M47.812
Tobacco use	Z72.0

MEDICATIONS

100 most recent outpatient medications released by VA to Veteran in the last 6 months

Medication Name and Dose	Quantity	Refill Number	Issue and Fill Date	Status
LEVOTHYROXINE NA (SYNTHROID) 150MCG TAB TAKE ONE TABLET BY MOUTH EVERY MORNING FOR HYPOTHYROIDISM	Qty:90	Fill: 1 of 2	Orig: 2024-01-11 Last: 2024-01-22	ACTIVE
LOSARTAN 50MG TAB TAKE ONE TABLET BY MOUTH EVERY DAY FOR BLOOD PRESSURE/HEART/KIDNEY. *DO NOT CUT IN HALF. DOSE IS 50MG DAILY*	Qty:30	Fill: 1 of 11	Orig: 2024-02-07 Last: 2024-03-11	ACTIVE
MELOXICAM 7.5MG TAB TAKE ONE TABLET BY MOUTH EVERY DAY FOR PAIN	Qty:90	Fill: 1 of 0	Orig: 2024-01-11 Last: 2024-01-19	ACTIVE
MULTIVITS W/MINERALS TAB/CAP (NO VIT K) TAKE 1 TABLET BY MOUTH EVERY DAY FOR VITAMIN SUPPLEMENT	Qty:100	Fill: 1 of 3	Orig: 2024-01-11 Last: 2024-01-11	ACTIVE
OMEPRAZOLE 20MG EC CAP TAKE ONE CAPSULE BY MOUTH EVERY MORNING BEFORE MEAL FOR HEARTBURN AND REFLUX. TAKE 30 TO 60 MINUTES BEFORE MEAL	Qty:90	Fill: 1 of 3	Orig: 2024-01-11 Last: 2024-01-21	ACTIVE
SERTRALINE HCL 100MG TAB TAKE TWO TABLETS BY MOUTH EVERY DAY FOR MOOD	Qty:180	Fill: 1 of 3	Orig: 2024-02-20 Last: 2024-02-20	ACTIVE

NON-VA MEDICATIONS

Local Drug Name and Dose	Medication Route	Schedule
ASPIRIN 325MG TAB	MOUTH	DAILY
ASPIRIN 81MG EC TAB	MOUTH	EVERY DAY

ALLERGIES

Name	Origin	Verified
PRAVASTATIN	Origin: 2013-04-04	Verified: 2013-04-04

ULTRAM 50MG TABLET

Origin: 2011-08-23

Verified: 2011-08-23

PROGRESS NOTES

LOCAL TITLE: VOPC PROVIDER NOTE

STANDARD TITLE: PRIMARY CARE NOTE

DATE OF NOTE: JAN 11, 2024@08:40 ENTRY DATE: JAN 11, 2024@08:40:48

AUTHOR: COLLIS,LAURIE C EXP COSIGNER:

URGENCY: STATUS: COMPLETED

*** VOPC PROVIDER NOTE Has ADDENDA ***

DATE: JAN 11, 2024

KELLEY,MARVIN STEPHEN, ***_**_****

Age: 73 Sex: MALE Race: WHITE

Weight: 199 lb [90.26 kg] (07/17/2023 08:59) Height: 72 in [182.9 cm]
(07/17/2023 08:59)Respiration: 18 (07/17/2023 08:59) Temperature: 96.5 F [35.8 C] (07/17/2023
08:59)

Pulse: 64 (07/17/2023 08:59) Blood Pressure: 134/64 (07/17/2023 08:59)

Pain Scale: 1 (07/17/2023 08:59)

DISCLAIMER: For patients being seen at DOD, Cerner Sites, and VHA facilities,
Remote Medications, Remote allergies and adverse drug reactions (ADRs) may be
incomplete. Patient medication list and allergies must be viewed using Joint
Legacy Viewer (JLV). For patients seen at DOD, Cerner Sites and VHA facilities,
I have reviewed patient medications using JLV.

ALLERGIES REMOTE AND LOCAL REVIEW:

FACILITY ALLERGY/ADR

AUDIE L. MURPHY MEMORIAL HOSP PRAVASTATIN
AUDIE L. MURPHY MEMORIAL HOSP ULTRAM 50MG TABLET
MICHAEL E. DEBAKEY VA MEDICAL CENTER NO KNOWN ALLERGIES

Remote and Local Allergies reviewed, and reactions confirmed.

Medications:

Active Inpatient, Outpatient and Clinic Medications (including Supplies):

	Status	Issue Date	Last Fill
Active Outpatient Medications	Refills	Expiration	
=====			
1) CYANOCOBALAMIN 1000MCG TAB Qty: 100 for ACTIVE		Issu:01-10-23	
90 days Sig: TAKE ONE TABLET BY MOUTH Refills: 3		Last:01-10-23	
EVERY DAY FOR DIETARY SUPPLEMENT		Expr:01-11-24	
2) GABAPENTIN 300MG CAP Qty: 30 for 30 ACTIVE		Issu:01-10-23	
days Sig: TAKE ONE CAPSULE BY MOUTH Refills: 3		Last:01-10-23	
AT BEDTIME FOR PAIN		Expr:01-11-24	
3) LEVOTHYROXINE NA (SYNTHROID) 150MCG TAB ACTIVE		Issu:07-17-23	
Qty: 90 for 90 days Sig: TAKE ONE Refills: 1		Last:11-03-23	
TABLET BY MOUTH EVERY MORNING FOR		Expr:07-17-24	

HYPOTHYROIDISM

- 4) LIDOCAINE 5% PATCH Qty: 60 for 30 days ACTIVE Issu:01-10-23
 Sig: APPLY 2 PATCHES TO SKIN EVERY DAY Refills: 6 Last:01-10-23
 FOR PAIN **PATCH MAY BE WORN FOR UP TO Expr:01-11-24
 12 HOURS IN A 24 HOUR PERIOD**
- 5) LOSARTAN POTASSIUM 50MG TAB Qty: 30 ACTIVE Issu:01-10-23
 for 30 days Sig: TAKE ONE TABLET BY Refills: 4 Last:12-27-23
 MOUTH EVERY DAY FOR BLOOD Expr:01-11-24
 PRESSURE/HEART/KIDNEY. *DO NOT CUT IN
 HALF. DOSE IS 50MG DAILY*
- 6) MELOXICAM 7.5MG TAB Qty: 90 for 90 days ACTIVE Issu:10-31-23
 Sig: TAKE ONE TABLET BY MOUTH EVERY Refills: 0 Last:10-31-23
 DAY FOR PAIN Expr:01-29-24
- 7) OMEPRAZOLE 20MG EC CAP Qty: 90 for 90 ACTIVE Issu:10-31-23
 days Sig: TAKE ONE CAPSULE BY MOUTH Refills: 0 Last:10-31-23
 EVERY MORNING BEFORE MEAL FOR Expr:01-29-24
 PREVENTION OF STOMACH ULCER (TAKE 30
 TO 60 MINUTES BEFORE MEAL; SHORT TERM
 THERAPY 90 DAYS) PLEASE TAKE WHILE
 TAKING ANY ANTI-INFLAMMATORY (NSAID)
 MEDICATIONS.
- 8) SERTRALINE HCL 100MG TAB Qty: 180 for ACTIVE Issu:01-10-23
 90 days Sig: TAKE TWO TABLETS BY Refills: 2 Last:09-22-23
 MOUTH EVERY DAY FOR MOOD Expr:01-11-24

Active Non-VA Medications	Start Date	Refills	Expiration
=====			
1) Non-VA ASPIRIN 81MG EC TAB	Sig: 81MG	ACTIVE	
MOUTH EVERY DAY			

9 Total Medications

Medication Reconciliation:

I have performed med rec with the patient/caregiver utilizing the Essential Medication List for Review (EMLR) MRT1. This included, under MRR1 active and pending prescriptions dispensed from local and remote VA prescriptions, DoD prescriptions, local non-VA medications, local inpatient medications orders, clinic medication orders and prescriptions that have expired or been discontinued in the past 90 days. There were no relevant meds aside from those listed above.

Active Medication List:

INCLUDED IN THIS LIST: Alphabetical list of active outpatient prescriptions dispensed from this VA (local) and dispensed from another VA or DoD facility (remote) as well as inpatient orders (local pending and active), local clinic medications, locally documented non-VA medications, and local prescriptions that have expired or been discontinued in the past 90 days.

Non-VA Meds Last Documented On: Oct 25, 2012

NOTE The display of VA prescriptions dispensed from another VA or DoD facility (remote) is limited to active outpatient prescription entries matched to National Drug File at the originating site and may not include some items such as investigational drugs, compounds, etc.

NOT INCLUDED IN THIS LIST: Medications self-entered by the patient into personal health records (i.e. My HealtheVet) are NOT included in this

list. Non-VA medications documented outside this VA, remote inpatient orders (regardless of status) and remote clinic medications are NOT included in this list. The patient and provider must always discuss medications the patient is taking, regardless of where the medication was dispensed or obtained.

Non-VA ASPIRIN 81MG EC TAB

TAKE ONE TABLET BY MOUTH EVERY DAY Patient wants to buy from Non-VA pharmacy.

OUTPT BUDESONIDE 160/FORMOTER 4.5MCG 120D INH (Status = Discontinued)
INHALE 2 PUFFS BY ORAL INHALATION TWICE A DAY **RINSE MOUTH WITH WATER**

Rx# 33381255 Last Released: 3/21/23 Qty/Days Supply: 1/30

Rx Expiration Date: 3/16/24 Refills Remaining: 11

OUTPT CYANOCOBALAMIN 1000MCG TAB (Status = Active)

TAKE ONE TABLET BY MOUTH EVERY DAY FOR DIETARY SUPPLEMENT

Rx# 25532150E Last Released: 1/11/23 Qty/Days Supply: 100/90

Rx Expiration Date: 1/11/24 Refills Remaining: 3

OUTPT GABAPENTIN 300MG CAP (Status = Active)

TAKE ONE CAPSULE BY MOUTH AT BEDTIME FOR PAIN

Rx# 32233032C Last Released: 1/12/23 Qty/Days Supply: 30/30

Rx Expiration Date: 1/11/24 Refills Remaining: 3

OUTPT LEVOTHYROXINE NA (SYNTHROID) 150MCG TAB (Status = Active)

TAKE ONE TABLET BY MOUTH EVERY MORNING FOR HYPOTHYROIDISM

Rx# 25725240 Last Released: 11/4/23 Qty/Days Supply: 90/90

Rx Expiration Date: 7/17/24 Refills Remaining: 1

Indication: FOR HYPOTHYROIDISM

OUTPT LIDOCAINE 5% PATCH (Status = Active)

APPLY 2 PATCHES TO SKIN EVERY DAY FOR PAIN **PATCH MAY BE WORN FOR UP TO 12 HOURS IN A 24 HOUR PERIOD**

Rx# 25698801 Last Released: 1/13/23 Qty/Days Supply: 60/30

Rx Expiration Date: 1/11/24 Refills Remaining: 6

Indication: FOR PAIN

OUTPT LOSARTAN POTASSIUM 50MG TAB (Status = Active)

TAKE ONE TABLET BY MOUTH EVERY DAY FOR BLOOD

PRESSURE/HEART/KIDNEY. *DO NOT CUT IN HALF. DOSE IS 50MG DAILY*

Rx# 25590537B Last Released: 12/30/23 Qty/Days Supply: 30/30

Rx Expiration Date: 1/11/24 Refills Remaining: 4

OUTPT MELOXICAM 15MG TAB (Status = EXPIRED)

TAKE ONE TABLET BY MOUTH ONCE A DAY FOR PAIN

Rx# 43069233A Last Released: 7/19/23 Qty/Days Supply: 90/90

Rx Expiration Date: 10/15/23 Refills Remaining: 0

Indication: FOR PAIN

OUTPT MELOXICAM 7.5MG TAB (Status = Active)

TAKE ONE TABLET BY MOUTH EVERY DAY FOR PAIN

Rx# 33590472 Last Released: 11/2/23 Qty/Days Supply: 90/90

Rx Expiration Date: 1/29/24 Refills Remaining: 0

Indication: FOR PAIN

OUTPT OMEPRAZOLE 20MG EC CAP (Status = Active)

TAKE ONE CAPSULE BY MOUTH EVERY MORNING BEFORE MEAL FOR PREVENTION OF STOMACH ULCER (TAKE 30 TO 60 MINUTES BEFORE MEAL; SHORT TERM THERAPY 90 DAYS) PLEASE TAKE WHILE TAKING ANY ANTI-INFLAMMATORY (NSAID) MEDICATIONS.

Rx# 33590473 Last Released: 11/1/23 Qty/Days Supply: 90/90

Rx Expiration Date: 1/29/24 Refills Remaining: 0

Indication: FOR PREVENTION OF STOMACH ULCER

OUTPT SERTRALINE HCL 100MG TAB (Status = Active)

TAKE TWO TABLETS BY MOUTH EVERY DAY FOR MOOD

Rx# 32011134D Last Released: 9/23/23 Qty/Days Supply: 180/90

Rx Expiration Date: 1/11/24 Refills Remaining: 2

SUPPLIES

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MEDICATION REVIEW

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I have reviewed this list of medications with the patient/caregiver and medication reconciliation was performed with discrepancy identified:

Problem List:

Active problems - Computerized Problem List is the source for the following:

1. Exposure to Potentially Hazardous Substance (SCT 133261000119105)
2. Cobalamin deficiency
3. Calculus of bile duct
4. HTN - Hypertension (SCT 38341003)
5. Noncompliance with treatment
6. Raised seborrheic keratosis
7. Primary malignant neoplasm of prostate
8. High risk drug monitoring status
9. Arthritis of left wrist
10. Myalgia
11. Cervical spondylosis with myelopathy
12. Cystitis
13. Abscess of axilla
14. Low back pain (SNOMED CT 279039007)
15. Cervicalgia (SNOMED CT 81680005)
16. Hyperlipidemia (SNOMED CT 55822004)
17. Obstructive sleep apnea (SNOMED CT 78275009)
18. Tobacco use
19. Chronic obstructive lung disease
20. Gastroesophageal reflux disease
21. Chronic post-traumatic stress disorder (SNOMED CT 313182004)
22. Hypothyroid (SNOMED CT 40930008)

Chief Complaint: follow up for labs and wanting a dermatology consult for skin changes

HPI/EVENTS SINCE LAST VISIT: In the last 6 months veteran has not had any ED visits, hospitalizations, or surgeries.

Veteran is pleasant and friendly. Veteran last visit was due to fall and having shoulder pain which has resolved by seeing ortho and having injections- stopped PT felt it did not help.

Veteran has skin changes to the face where he is outside due to hunting and fishing- would like a derm consult.

SOCIAL HISTORY:

Tobacco Use: No
Alcohol Use: No
Illicit Drug Use: Yes. MJ

Review of Systems:

- General: no fevers, no chills, no fatigue, no anorexia, no night sweats, no weakness, no malaise, no unintentional weight loss
- Eyes: no decreased vision, no eye pain, + glasses
- Ears: no discharge, + tinnitus, no vertigo, no hearing loss
- Resp: no cough, no sputum, no hemoptysis, no wheezing, no pain.
- CV/PV: no chest pain, no palpitations, no DoE, no murmur, no syncope, no varicose veins, no swelling in calves, legs, or feet
- MSK: + R shoulder joint pain, no joint swelling, no heat, no erythema, no stiffness, no deformities, no muscle wasting
- Endo: no polyuria, no polydipsia, no polyphagia,
- Neuro: no changes in mood, attention, or speech; no changes in orientation, memory, insight, or judgment, no headache, no vertigo, no tremor, no seizures, no weakness, no numbness or tingling
- Psych: no si/hi, no nervousness or anxiety, no depression, +PTSD

INTERIM HISTORY:

PHYSICAL EXAMINATION:

Temp: 97.8 F [36.6 C] (01/11/2024 08:46)
P: 54 (01/11/2024 08:46)
R: 18 (01/11/2024 08:46)
B/P: 136/74 (01/11/2024 08:46)
Weight: 196.9 lb [89.31 kg] (01/11/2024 08:46)
Height: 72 in [182.9 cm] (01/11/2024 08:46)
BMI:26.8

General: No acute distress, AAOx4, appropriate for situation and vitals signs noted
HEENT: Normocephalic, atraumatic, +EOM
Neck: Supple, trachea midline, No JVD or masses, no caortid bruits
Lymph nodes: No cervical adenopathy
Cardio: Regular rate, regular rhythm, S1 S2 noted
Respiratory: Bil BS clear upon auscultation, resp even and unlabored, able to speak full sentences
Abdomen: Soft, nontender, nondistended, bowel sounds present x4 quad
Extremities: No cyanosis, swelling or deformity noted
Psych: Calm, cooperative, no suicidal or homicidal ideation, normal affect
Nuero: No focal deficits, CNS 1-12 intact
Skin: Veteran has facial nevi and some area with skin changes to the bil cheeks

LABS:

CBC:

Collection DT	Specimen	Test Name	Result	Units	Ref Range
01/04/2024 08:51	BLOOD	WBC	5.9	10.e3/uL	4.0 - 10.0
01/04/2024 08:51	BLOOD	RBC	4.40 L	10.e6/uL	4.5 - 5.9
01/04/2024 08:51	BLOOD	HGB	12.8 L	g/dL	13.5 - 17.5
01/04/2024 08:51	BLOOD	HCT	39.8 L	%	41.0 - 53.0
01/04/2024 08:51	BLOOD	MCV	90.4	fL	78.0 - 98.0
01/04/2024 08:51	BLOOD	MCH	29.1	pg	26.0 - 34.0
01/04/2024 08:51	BLOOD	MCHC	32.2	gm/dL	32.0 - 36.0
01/04/2024 08:51	BLOOD	PLTS	279	10.e3/uL	150 - 400

Chem 20:

Date	Test	Result	Units	Normal Range	Specimen
01/04/2024	CREATININE	1.0	mg/dL	.7 - 1.3	SERUM
01/04/2024	UREA NITROGEN	20	mg/dL	7 - 25	SERUM
01/04/2024	GLUCOSE	106 H	mg/dL	70 - 105	SERUM
01/04/2024	SODIUM	143	mmol/L	136 - 145	SERUM
01/04/2024	POTASSIUM	4.7	mmol/L	3.5 - 5.1	SERUM
01/04/2024	CHLORIDE	104	mmol/L	98 - 107	SERUM
01/04/2024	CALCIUM	8.9	mg/dl	8.6 - 10.3	SERUM
01/04/2024	PROTEIN,TOTAL	6.7	g/dL	6.4 - 8.9	SERUM
01/04/2024	ALBUMIN	4.3	g/dl	3.5 - 5.7	SERUM
01/04/2024	TOT. BILIRUBIN	0.3		0.3 - 1.0	SERUM
01/04/2024	ALKALINE PHOSP	52	U/L	34 - 104	SERUM
01/04/2024	AST	14	IU/L	13 - 39	SERUM
01/04/2024	ALT	16	IU/L	7 - 52	SERUM
01/04/2024	ANION GAP	10.0		6 - 15	SERUM
01/04/2024	CO2	29.0	mmol/L	21 - 31	SERUM
01/04/2024	ICTERUS	0	NEG		SERUM
01/04/2024	LIPEMIA	0	NEG		SERUM
01/04/2024	HEMOLYSIS	0	NEG		SERUM
01/04/2024	OSMOLALITY-CAL	288.00	mOsm/kg		SERUM
01/07/2022	zzEGFR	>60			PLASMA
01/04/2024	EGFR CKD EPI	79			SERUM

A1C:

Date	Test	Result	Units	Normal Range	Specimen
01/04/2024	HEMOGLOBIN A1C	6.0	%	4.0 - 6.0	BLOOD

PROSTATIC SPECIFIC ANTIGEN 1/4/24 08:51 0.01 L

LIPID PROFILE Coll. dat1/4/24 08:51 9/19/23 08:44 7/10/23 09:20

CHOL	177	159	210 H
TRIG	83	125	163
HDL	52	46	48
LDL CHOLESTEROL	108.00	88.00	129.00
VLDL CHOL	17.0	25.0	33.0
LDL DIRECT			

ASSESSMENT/PLAN:

Skin changes to the face- CC derm consult placed

Hypothyroid. increase Synthroid dose to 150 from last vist and TSH now is WNL,

reinforced to take medication alone with just water, plan to recheck TSH in 2 months.

R Shoulder pain- resolved with ortho referral from last visit.

HTN - BP in clinic 130/60's Veteran stats he does not check is BP at home- educated on checking BP at home. Stats he is taking medication as prescribed. Educated pt on goal BP and negative outcomes related to target organs when BP is uncontrolled. Educated on low na and DASH diet.

Asthma - no change; h/o Symbicort d/c'd 2/2 non-use; guaifenesin prn; CC PULM Dr. Samuel Copeland reports has had appt and placed on Symbicort which he gets non va educated to see with Dr. Copeland on upcoming visit in order to submit to CC Rx for refill of medicatin.

B12 deficiency - Lab WNL, taking supplement as prescribed.

anemia - mild; chronic. Handout on foods high in iron given- Educated on dietary sources of iron. CTM

allergic rhinitis - antihistamine/nasal rinse, reports effective symptom control.

OSA - f/u with sleep clinic PRN.

hearing issues- place a CC consult for audiology.

Prostate cancer - PSA marginal s/p prostatectomy 10/2021, CC Urology Dr. Aaron New- no issues reported on this visit.

constipation - stool softener; drink lots of water, increase fiber/exercise

insomnia - reports no issues on this visit.

PTSD - denies SI/HI, controlled on current therapy, declines referral with MH; crisis hotline card offered; VET aware of MH walk-in policy- veteran has diversional activities like hunting and fishing.

colorectal cancer screening - 2021 + for diverticulosis, reports no concerns or symptoms today.

Return to clinic in 6 months

Non fasting labs prior to visit- CBC, CMP, Hgb A1C, UA, Lipid panel, PSA, TSH, VitD and Vit B.

/es/ LAURIE C COLLIS

DNP

Signed: 01/11/2024 12:21

02/20/2024 ADDENDUM

STATUS: COMPLETED

Pt is wanting another derm consult to be put back in. Please advise.

/es/ MONIQUA J FULLER

AMSA

Signed: 02/20/2024 13:12

Department of Veterans Affairs		REQUEST FOR SERVICES (RFS) FORM	
PREVIOUS AUTHORIZATION NUMBER: TODAY'S DATE (MM/DD/YYYY):		NOTE: The Request for Services (RFS) Form 10-10172 must be submitted via an approved method (HSRM, Electronic fax, Direct Messaging, traditional fax, or mail). Completion of this form is REQUIRED and MUST BE SIGNED by the requesting provider for further care to be rendered to a Veteran patient.	
SECTION I: VETERAN INFORMATION			
1. VETERAN'S LEGAL FULL NAME (First, MI, Last):		2. DOB (MM/DD/YYYY):	
3. VA FACILITY: STVHCS/Pink Team - Team Fax: (210) 443-0301		4. VA LOCATION: San Antonio, TX Ph: (210) 949-3850	
SECTION II: ORDERING PROVIDER INFORMATION			
5. REQUESTING PROVIDER'S NAME:		6. NPI #:	7. SPECIALTY:
8. OFFICE NAME & ADDRESS:			
9. SECURE EMAIL ADDRESS:			
10. PHONE NUMBER:	11. FAX NUMBER:		☐ 12. INDIAN HEALTH SERVICES (IHS) PROVIDER?
SECTION III: TYPE OF CARE REQUEST			
13. PLEASE INDICATE CLINICAL URGENCY (Urgent care is only applicable for requests that require less than 3 days to process. If care is needed within 48 hours or if Veteran is at risk for Suicide/Homicide, please call the VA directly on the same day as completed RFS form submission. Do NOT mark urgent for administrative urgency): ☐ ROUTINE ☐ URGENT			
14. DIAGNOSIS (ICD-10 Code/Description):		15. DATE OF SERVICE (MM/DD/YYYY) &/OR ANTICIPATED LENGTH OF CARE:	
16. CPT/HCPCS CODE &/OR DESCRIPTION OF REQUESTED SERVICES (Include units/visits, add second list page, if needed):			
17. HOW MANY VISITS HAVE OCCURRED SO FAR? (If known)		18. IS THIS A REFERRAL TO ANOTHER SPECIALTY? ☐ YES (If "YES," please fill out the Servicing Provider/Specialty information below) ☐ NO	
19. SERVICING PROVIDER'S NAME:		20. NPI #:	21. SPECIALTY:
22. OFFICE NAME & ADDRESS:			
23. SECURE EMAIL ADDRESS:			
24. PHONE NUMBER:		25. FAX NUMBER:	
SECTION IV: TYPE OF SERVICE REQUESTED			
26. OUTPATIENT CARE: ☐ PT ☐ OT ☐ SPEECH THERAPY		27. SURGICAL PROCEDURE: ☐ INPATIENT ☐ OUTPATIENT	
FREQUENCY & DURATION:		FACILITY NAME:	
28. ☐ IN-OFFICE PROCEDURE		29. INPATIENT CARE: ☐ LTACH ☐ ACUTE REHAB ☐ BH	
30. ☐ ADDITIONAL OFFICE VISITS (List # needed):		31. ☐ EXTENSION OF VALIDITY DATES	
32. ☐ EMERGENCY ROOM CARE		33. ☐ LABS (If done outside of office, please provide facility above)	
34. ☐ RADIOLOGY/IMAGING (If done outside of office, please provide facility above)		35. ☐ PRE-OPS LABS ☐ CHEST XRAY ☐ EKG ☐ OTHER:	
36. JUSTIFICATION FOR REQUEST (To avoid delays in care, include appropriate documentation such as office notes, current treatment plans, clinical history, laboratory results, radiology results &/or medications to support the medical necessity of services requested).			

VETERAN'S LEGAL FULL NAME (First, MI, Last):		
SECTION V: GERIATRICS AND EXTENDED CARE SERVICES (If applicable)		
37. ☐ COMMUNITY ADULT DAY HEALTH CARE ☐ COMMUNITY NURSING HOME ☐ HOMEMAKER/HOME HEALTH AIDE ☐ HOME INFUSION ☐ HOSPICE/PALLIATIVE CARE ☐ RESPITE ☐ SKILLED HOME HEALTH CARE ☐ OTHER: _____		
FREQUENCY & DURATION: _____		
38. JUSTIFICATION FOR REQUEST (To avoid delays in care, include appropriate documentation such as office notes, current treatment plans, clinical history, laboratory results, radiology results &/or medications to support the medical necessity of services requested).		
SECTION VI: HOME OXYGEN INFORMATION (If applicable)		
39. PAO2 AT REST:	40. O2 SAT AT REST:	41. OXYGEN FLOW RATE:
42. EXTENT OF SUPPORT (Continuous, Intermittent, Specific Activity):		
43. OXYGEN EQUIPMENT (Stationary/Portable):		
44. DELIVERY SYSTEM (Cannula, Mask, Other):		
SECTION VII: DME & PROSTHETICS INFORMATION (If applicable)		
45. HCPS FOR THE ITEM(S) BEING PRESCRIBED:		
46. BRAND, MAKE, MODEL, PART NUMBERS:		
47. MEASUREMENTS:		
48. QUANTITY:	49. ICD-10:	50. PROVISIONAL DIAGNOSIS:
51. DELIVERY/PICKUP OPTIONS: ☐ DELIVER TO ORDERING PROVIDER'S ADDRESS ☐ VETERAN WILL PICKUP AT THE VA MEDICAL CENTER ☐ DELIVER TO COMMUNITY VENDOR FOR DELIVERY & SETUP FOR DME ☐ DELIVER TO VETERAN'S HOME		
SECTION VIII: DURABLE MEDICAL EQUIPMENT (DME) EDUCATION & TRAINING (If applicable)		
Please see DME Requirements/Pharmacy Requirements - Community Care (va.gov) for URGENT DME requests.		
NOTE: Failure to thoroughly complete the RFS for DME will result in delayed patient care & prevent the VA from DME fulfillment.		
52. BEFORE DME WILL BE ISSUED, EDUCATION, TRAINING, &/OR FITTING OF DME (as applicable for the specific DME being ordered) TO THE VETERAN MUST BE COMPLETE. PLEASE INDICATE WHETHER THE FOLLOWING HAS BEEN COMPLETED FOR THE VETERAN. NOTE: If not completed, DME will be mailed to requesting provider's address to coordinate an alternative time for proper instruction on DME use.		A. EDUCATION: ☐ YES ☐ NO B. TRAINING: ☐ YES ☐ NO ☐ N/A C. FITTING: ☐ YES ☐ NO ☐ N/A
53. JUSTIFICATION FOR REQUEST (To avoid delays in care, include appropriate documentation such as office notes, current treatment plans, clinical history, laboratory results, radiology results &/or medications to support the medical necessity of services requested).		

VETERAN'S LEGAL FULL NAME (First, MI, Last):	
SECTION IX: THERAPEUTIC FOOTWEAR ASSESSMENT INFORMATION (If applicable)	
54. FILL OUT THE INFORMATION BELOW (If applicable): ☐ LEFT FOOT ☐ RIGHT FOOT ☐ BILATERAL ☐ PREFABRICATED THERAPEUTIC FOOTWEAR ☐ CUSTOM THERAPEUTIC FOOTWEAR NOTE: For prescription of therapeutic footwear for severe or gross foot deformity which cannot be accommodated with conventional footwear. DESCRIBE FOOT DEFORMITY AND ADDITIONAL DETAILS:	NOTE: For prescription of therapeutic footwear due to disease pathology resulting in neuropathy or peripheral artery disease. 55. CHECK APPROPRIATE DIABETIC/AMPUTATION RISK SCORE: ☐ RISK SCORE 2: PATIENT DEMONSTRATED SENSORY LOSS (<i>inability to perceive the Semmes-Weinstein 5.07 monofilament</i>), DIMINISHED CIRCULATION AS EVIDENCED BY ABSENT OR WEAKLY PALPABLE PULSES, FOOT DEFORMITY, OR MINOR FOOT INFECTION, & A DIAGNOSIS OF DIABETES. ☐ RISK SCORE 3: PATIENT DEMONSTRATED PERIPHERAL NEUROPATHY WITH SENSORY LOSS (<i>i.e., inability to perceive the Semmes-Weinstein 5.07 monofilament</i>), AND DIMINISHED CIRCULATION, AND FOOT DEFORMITY, OR MINOR FOOT INFECTION & A DIAGNOSIS OF DIABETES, OR ANY OF THE FOLLOWING BY ITSELF: (1) PRIOR ULCER, OSTEOMYELITIS OR HISTORY OF PRIOR AMPUTATION; (2) SEVERE PERIPHERAL VASCULAR DISEASE (PVD) (<i>intermittent claudication, dependent rubor with pallor on elevation, or critical limb ischemia manifested by rest pain, ulceration or gangrene</i>); (3) CHARCOT'S JOINT DISEASE WITH FOOT DEFORMITY; & (4) END STAGE RENAL DISEASE. NOTE: Only patients who are experiencing medical conditions noted in the risk scores can be prescribed therapeutic/diabetic footwear.
*ATTESTATION: I do hereby attest that the forgoing information is true, accurate, & complete to the best of my knowledge & I understand that any falsification, omission, or concealment of material fact may subject me to administrative, civil, or criminal liability. I do hereby acknowledge that VA reserves the right to perform the requested service(s) if the following criteria are met: (1) The patient agrees to receive services from VA (2) Service(s) are available at VA facility & are able to be provided by the clinically indicated date (3) It is determined to be within the patients best interest. Upon completion of the requested service(s), VA will provide all resulting medical documentation to the ordering provider. If all criteria listed are not true & VA agrees the service(s) are clinically indicated, VA will provide a referral for services to be performed in the community. I do hereby attest that upon receipt of order/consult results, I will assume responsibility for reviewing said results, addressing significant findings, & providing continued care.	
56. REQUESTING PROVIDER SIGNATURE (Required):	57. TODAY'S DATE (MM/DD/YYYY):

To facilitate timely review of this request, the most recent office notes & plan of care must accompany this signed form.

For more information please visit: https://www.va.gov/COMMUNITYCARE/providers/Care_Coordination.asp.

VA Community Care Medical Policies describe standard VA health care benefit for services and procedures that community providers may recommend as necessary for a Veteran. Prior to providing care, providers should use the Community Care Medical Policies as a reference when determining if a Veteran meets VA clinical criteria. When additional services are requested, Community Care Medical Policies will be used to determine approval by a clinical reviewer. Community Care Medical Policies & supporting information can be found at: <https://www.va.gov/COMMUNITYCARE/providers/Medical-Policy.asp>